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Claranta 500

Clarithromycin Tablets USP 500 mg

Composition:

Each film coated tablet contains:
Clarithromycin USP 500 mg

Dosage Form:

Film coated tablet

Description:

Yellow coloured capsule shaped film coated tablets having bisect line on one side.

Mode of Action:

Claranta tablets contain clarithromycin that is a macrolide antibiotic. CLARANTA exerts bactericidal or bacteriostatic activity depending on the strains of organisms.

The action of CLARANTA, like that of other macrolides, is due to its reversible binding with the 50 S ribosomal subunit within the bacterial cell. This results in inhibition of the translocation of aminoacyl transfer-RNA and protein synthesis. CLARANTA is taken up by polymorphs, by an active uptake and enters into lymphocytes, polymorphs and macrophages. It has also been shown to further enhance the natural killer T-cell activity, already elevated in the presence of viral pathogens.

Pharmacokinetic Properties:

CLARANTA is rapidly absorbed from the gastrointestinal tract after oral administration. Food slightly delays both the onset of CLARANTA absorption and the antimicrobially active metabolite, 14-OH Clarithromycin, but does not affect the extent of bioavailability. In fasting healthy human subjects, peak serum concentrations are attained within 2 hours after oral dosing. Steady state peak serum Clarithromycin concentrations are attained in 2 to 3 days and are approximately 1ug/ml with a 250 mg dose administered every 12 hours, 2 to 3 ug/ml with a 500 mg dose administered every 12 hours and 3 to 4 ug/ml with a 500 mg dose administered every 8 hours. The elimination half-life of Clarithromycin is about 3 to 4 hours with 250 mg administered every 12 hours but increases to 5 to 7 hours with 500 mg administered every 8 to 12 hours. After 250 mg tablet every 12 hours, approximately 20% of the dose is excreted in the urine as Clarithromycin while after a 500 mg tablet every 12 hours, the urinary excretion of Clarithromycin is somewhat greater, approximately 30 %.

Therapeutic Indications:

CLARANTA tablets are indicated for the treatment of the infections caused by organisms susceptible to it. Such infectious include:

- (i) Upper respiratory tract infections e.g. Sinusitis, Pharyngitis
- (ii) Lower respiratory tract infections e.g. Bronchitis, pneumonia
- (iii) Skin and soft tissue infections e.g. Folliculitis, cellulitis erysipelas.
- (iv) Disseminated or localised infections caused by atypical mycobacteria, legionella and Chlamydia.

Dosage and Method of Administration:

The usual dose of CLARANTA for adults is 250 to 500 mg twice daily given orally for 7 to 14 days. For children 7 - 12 years a dose of 7.5 mg/kg twice daily has been suggested. CLARANTA can be given before, with or after a meal.

In patients with renal impairment (creatinine clearance less than 30 ml/min) the dose may be reduced to 250 mg once a day or 250 mg twice a day in severe infections.

Mode of administration: Oral

Contraindications:

CLARANTA is contraindicated in patients with a known hypersensitivity to Clarithromycin, Erythromycin, or any of the macrolide antibiotics. Concomitant administration of Clarithromycin with Cisapride, Pimozide or Terfenadine is contraindicated. There have been post-marketing reports of drug interactions when Clarithromycin is co-administered with Cisapride, Pimozide or Terfenadine resulting in cardiac arrhythmias most likely due to inhibition of hepatic metabolism of these drugs by CLARANTA. Fatalities have been reported.

Concomitant administration of Clarithromycin and the following drugs is contraindicated: Domperidone as this may result in QT prolongation and cardiac arrhythmias including ventricular tachycardia, ventricular fibrillation, and torsades de pointes.

Warning and Precautions:

Clarithromycin is principally excreted by the liver and kidney. Caution should be exercised in administering this antibiotic to patients with impaired hepatic or renal function. Prolonged or repeated use of clarithromycin may result in an overgrowth of non-susceptible bacteria or fungi. If superinfection occurs, clarithromycin should be discontinued and appropriate therapy instituted. H.pylori organisms may develop resistance to clarithromycin in a small number of patients.

In the event of severe acute hypersensitivity reactions, such as anaphylaxis, severe cutaneous adverse reactions (SCARs) [e.g. Stevens-Johnson Syndrome (SJS) toxic epidermal necrolysis (TEN), drug reaction with eosinophilia and systemic symptoms (DRESS) & acute generalised exanthematous pustulosis (AGEP)], CLARANTA should be discontinued immediately and appropriate treatment

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should be urgently initiated.

Interactions:

When Clarithromycin and Terfenadine were co-administered, plasma concentrations of the active acid metabolite of terfenadine were threefold higher, on average, than the values observed when terfenadine was administered alone. Concomitant administration of clarithromycin with terfenadine is contraindicated.

Increased Rifabutin toxicity has been reported in patients receiving Clarithromycin and Rifabutin and there has a report of delirium following concurrent use with Fluoxetine.

CLARANTA used in patients who are receiving Theophylline may be associated with an increase of serum theophylline concentrations. Concomitant administration of single dose of Clarithromycin and Carbamazepine has been shown to result in increased plasma concentrations of carbamazepine. Blood level monitoring of Carbamazepine may be considered.

Co-administration of Clarithromycin, known to inhibit CYP3A, and a drug primarily metabolized by CYP3A may be associated with elevations in drug concentrations that could increase or prolong both therapeutic and adverse effects of the concomitant drug. The following drugs or drug classes are known or suspected to be metabolized by CYP3A isozyme: Domperidone.

Pregnancy and Lactation:

The safety of clarithromycin during pregnancy and breast feeding of infants has not been established. CLARANTA should thus not be used during pregnancy or lactation unless the benefit is considered to outweigh the risk. Clarithromycin has been found in the milk of lactating animals and in human breast milk.

Undesirable Effects:

CLARANTA has generally been well tolerated causing only mild side effects. The most frequently observed in clinical studies have been diarrhoea, nausea, abnormal taste, dyspepsia, abdominal discomfort or pain and headache.

Skin and Subcutaneous Tissue Disorders

Frequency not known: severe cutaneous adverse reactions (SCARs) including Stevens-Johnson syndrome (SJS), toxic epidermal necrolysis (TEN), drug reaction with eosinophilia and systemic symptoms (DRESS) & acute generalised exanthematous pustulosis (AGEP).

Overdose:

Severe adverse reactions are unlikely in cases of overdosage with CLARANTA. Symptoms are likely to be related to the GI tract and may include nausea and vomiting. Management of overdosage is with gastric lavage and supportive measures.

Pack Size:

Each film coated tablet containing Clarithromycin USP 500mg is available in blister of 10 tablets. Available in a box of (10x1x10's).

Storage Conditions:

Store at a temperature below 30°C. Protect from light.

Shelf life:

36 months

Manufactured by:

ajanta pharma limited

B-4/5/6 MIDC Area, Paithan 431148

Dist. Aurangabad, India

Imported by : Jetpharma Sdn Bhd, No 13, Jalan Rajawali 2, Bandar Puchong Jaya, 47100 Puchong, Selangor.

Registration Holder:

GoPharma Sdn Bhd, Selangor, MALAYSIA

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